TD VACCINE

Wound management

Pregnancy: Tdap recommended

CHEMICAL NAME Adsorbed tetanus toxoid + reduced-dose diphtheria toxoid (formaldehyde-inactivated toxins on aluminium adjuvant)

CLASS Inactivated toxoid vaccine (active immunisation)

TRADE NAMES Td-Vac · Revaxis (Td/IPV) · Tenivac · Boostrix (Tdap)

THERAPEUTIC USES / INDICATIONS

- Primary immunisation and boosters against tetanus and diphtheria (adolescents, adults).
- Tetanus-prone wound management in incompletely immunised patients.
- Pregnancy (Tdap 16–32 weeks in many countries) to protect newborns from pertussis/tetanus.

ADMINISTRATION

- **IM** into deltoid (adults) or anterolateral thigh (infants); 0.5 mL.
- Shake well; store at 2–8 °C, do not freeze.
- Can be given with other vaccines at different sites.

DOSAGES

- **Primary course (unvaccinated adults):** 3 doses of 0.5 mL — 0, 1 month, 6–12 months; then boosters (per national schedule, e.g. every 10 years).
- **Wound management:** booster if >5–10 years since last dose (per tetanus-prone status) — with tetanus immunoglobulin if high-risk and incompletely immunised.
- **Pregnancy:** one Tdap dose each pregnancy.

PHARMACOKINETICS

- Protective antitoxin levels develop ~2 weeks after second/third dose; boosters give rapid anamnestic response.

PHARMACODYNAMICS

- Toxoids stimulate **antitoxin antibodies** (IgG) that neutralise tetanus and diphtheria toxins → active long-term immunity.

SIDE & ADVERSE EFFECTS

SIDE EFFECTS (COMMON)

- Injection-site pain, redness, swelling.
- Low-grade fever, malaise.
- Headache, myalgia.

ADVERSE EFFECTS (SERIOUS)

- Arthus-type reaction (extensive local swelling) with too-frequent boosters.
- Anaphylaxis (rare).
- Brachial neuritis, Guillain–Barré syndrome (very rare).

CONTRAINDICATIONS & CAUTIONS

- Anaphylaxis to previous dose or vaccine component.
- Acute febrile illness (postpone; minor illness not a reason to delay).
- Previous neurological complications after diphtheria/tetanus vaccine (caution).

KEY INTERACTIONS

- **Immunosuppressants:** reduced response.
- **Anticoagulants:** IM haematoma (use fine needle, firm pressure).

NURSING MANAGEMENT

- Check immunisation history; document batch number and site.
- Assess wound: tetanus-prone (soil, manure, puncture, devitalised tissue, >6 h delay).
- Observe for 15 min after vaccination; anaphylaxis kit available.
- Cold-chain storage.

PATIENT EDUCATION

- Sore arm and mild fever are common for 1–2 days — paracetamol helps.
- Keep your vaccination record; tetanus boosters as scheduled.
- Clean wounds well and seek care for dirty or deep wounds.

KEY POINTS

- Tetanus + diphtheria toxoid (active immunity).
- Wound management ± tetanus immunoglobulin.
- Primary 3 doses; boosters per schedule.
- Tdap in each pregnancy.

SOURCES CHECKED **[1004]** REVAXIS suspension for injection in pre-filled syringe (UK SmPC)

Full references p. 432–457